

Hepatitis B (Occupational)

Vaccination safety requirements

The signed PGD behind this page is unaltered. This correction takes precedence over it.

What is missing from the signed document

- No requirement for adrenaline, and no anaphylaxis provision of any kind.
- No cold chain excursion procedure.
- No sharps disposal provision, in a PGD whose whole purpose is occupational needlestick risk.

What must be in place before you vaccinate under this PGD

- Adrenaline (epinephrine) 1 in 1,000 injection must be immediately available in the room where vaccination takes place, in date, together with a telephone.
- A written anaphylaxis protocol consistent with current Resuscitation Council UK guidance must be available, and every person administering must be trained in the recognition and immediate management of anaphylaxis and hold current basic life support training.
- Observe every patient for 15 minutes after vaccination. Vaccinate seated, and have procedures in place to prevent injury from a faint.
- Record the batch number and the anatomical site for every dose given. A recall cannot be actioned without the batch number.

If these are not in place

Do not vaccinate under this PGD. This is not a documentation point: anaphylaxis after vaccination is rare, and it is the thing that kills. The requirement exists so that the one time it happens, the adrenaline is in the room.

Why this notice exists

A review of every vaccination PGD on the platform on 8 September 2026 found that these requirements were stated in some signed documents and not others. Where they were absent, nothing in the signed document required adrenaline to be present. Pharmacies almost certainly have it and are trained; the problem is that the document did not require it, so it was not evidenced anywhere if anyone asked.

A corrected version of this PGD will be issued and will carry these requirements in the body of the document. Until then, this notice applies.

This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction
for the administration of Engerix B for vaccination against Hepatitis B

Summary of NICE / NICE CKS Guidelines for Hepatitis B Vaccination

Overview

- **Hepatitis B** is a **vaccine-preventable viral infection** of the liver, transmitted through **blood and body fluids**.
- The **Hepatitis B vaccine** is a **recombinant subunit vaccine** (non-live) that provides effective long-term protection.

Routine and Targeted Vaccination

1. Universal Childhood Programme

- Since August 2017, **Hepatitis B is included in the 6-in-1 (DTaP/IPV/Hib/HepB) vaccine**.
- Schedule: **8, 12, and 16 weeks of age**, as part of routine UK childhood immunisation.

2. Targeted Vaccination for At-Risk Groups

Recommended for individuals at higher risk, including:

- **Babies born to Hepatitis B positive mothers**
- **Healthcare workers and care workers** at risk of exposure
- **People who inject drugs** or have high-risk sexual behaviour
- **Household/sexual contacts of HBV carriers**
- Individuals with **chronic kidney disease** (especially on dialysis)
- **Prisoners**
- **Travellers to high-prevalence countries**
- People with **liver disease, HIV, or immunosuppression**

Vaccination Schedules

Standard Schedule (Adults and children ≥ 1 year):

- **0, 1, and 6 months** (3 doses)
- A **booster may be given** at 5 years for ongoing risk, though long-term protection is expected.

Accelerated Schedule (For rapid protection – e.g. travel or post-exposure):

- **0, 1, and 2 months** with a booster at 12 months

Ultra-Rapid Schedule (e.g. travellers):

- **Day 0, 7, and 21**, with a booster at 12 months

Use **monovalent Hepatitis B vaccine** for adults or for birth doses in infants born to infected mothers.

Post-Vaccination Serology

- **Test for anti-HBs** (antibody to Hepatitis B surface antigen) 1–4 months after final dose **if high-risk**, e.g.:
 - Healthcare workers
 - Babies born to infected mothers
 - Immunocompromised individuals
 - **Anti-HBs ≥ 10 mIU/mL** = protective
 - If inadequate response: offer further doses (up to 6 total), then reassess.
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Contraindications and Precautions

- Severe allergic reaction to a previous dose or any component.
 - Acute severe illness (defer until recovery).
 - Safe in pregnancy and breastfeeding.
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Side Effects

- Usually mild: local pain, redness, low-grade fever.
 - Rare: hypersensitivity reactions.
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Documentation

- Record **brand, dose, batch number, route, site, and date.**
- Report adverse events via the **Yellow Card Scheme.**

Patient Group Direction

for the supply/administration of

Engerix B

for vaccination against

Hepatitis B

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	● Pharmacist registered and practicing with the GPhC ● Pharmacy technician registered and practicing with the GPhC
Training and assessment	● Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. ● All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines ● All users must previously have used PGD's to supply medication ● Must work in compliance with the SOPs of their own employer ● All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	● All users must be up to date with CPD requirements and appraisal ● All users must be up to date with MHRA and safety alerts with regards to medical products ● All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Engerix B is indicated for the active immunisation of individuals at risk of exposure to hepatitis B virus, including healthcare workers, infants born to infected mothers, and individuals at occupational or lifestyle risk.
Inclusion criteria	- Individuals of any age who are at increased risk of hepatitis B infection. - Eligible under national immunisation or occupational health guidance. - Informed consent obtained. - No contraindications to the vaccine.
Exclusion criteria	- Known hypersensitivity to the active substance or any excipients. - Previous allergic reaction to any hepatitis B vaccine. - Acute severe febrile illness (postpone until recovered).
Cautions	- Use with caution in individuals with bleeding disorders or on anticoagulants. - Observe patient for 15 minutes post-vaccination. - Ensure adherence to the complete vaccination schedule. - Provide follow-up advice and written patient information leaflet (PIL).
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate

Details of the medicine

Name, form and strength of medicine	Engerix B
Legal category	POM
Storage	Store in a refrigerator (2°C–8°C). Do not freeze. Protect from light.
Route/method of administration	Intramuscular injection, usually in the deltoid muscle.
Dose and frequency	Adults and adolescents (16+): 20 micrograms (1 mL) per dose. Standard schedule: 0, 1, and 6 months. Accelerated schedule: 0, 1, 2, and 12 months.
Quantity to be administered and/or supplied	1 mL per dose depending on the schedule.
Maximum or minimum treatment period	Typically 6–12 months depending on selected schedule.
Adverse effects	Injection site pain, redness, swelling, fatigue, headache, fever, gastrointestinal symptoms. Rare: allergic reactions. Refer to SmPC.

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: ● that valid informed consent was given ● name of individual, address, date of birth and GP ● name of healthcare practitioner ● name and brand of medication ● date of supply dose, form and route quantity supplied ● advice given, including advice given if excluded or declines treatment ● details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: ● Adults aged 18 years and over - keep records for 8 years ● Children aged under 18 years -keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with the medication.
Follow-up advice to be given to patient or carer	To seek medical advice if symptoms worsen rapidly or significantly, do not improve in 3–4 weeks, or they become systemically very unwell.

Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017 <https://www.nice.org.uk/guidance/mpg2>
- NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017 <https://www.nice.org.uk/guidance/mpg2/resources>
- NICE CKS Guidance
- British National Formulary (BNF) Key information on the selection, prescribing, dispensing and administration of medicines [BNF \(British National Formulary\) | NICE](#)
- Summary of Product Characteristics

Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises / clinic premises to which this PGD relates

Name of healthcare professional	Job title	Registration number	Signature
Valid from date		Expiry date	
25/8/26		31/7/27	

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or medicine.

This section must be signed by the superintendent or clinical lead responsible for this service.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date

Name of professional group signatory	Job title & name of organisation	Signature	Date
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Signed on behalf of Get Real Health

Name	Qualification	Signature	Date
Nitin Shori	Medical Doctor GMC: 6047293		25/8/2025
Chris Pilkington	Pharmacist GPhC: 2046322		25/8/2025

Change history

Version number	Change details	Date
001	Development & issue of new PGD	25 th June 2025

Patient Group Direction

for the supply/administration of

HBvaxPRO

for vaccination against

Hepatitis B

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	● Pharmacist registered and practicing with the GPhC ● Pharmacy technician registered and practicing with the GPhC
Training and assessment	● Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. ● All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines ● All users must previously have used PGD's to supply medication ● Must work in compliance with the SOPs of their own employer ● All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	● All users must be up to date with CPD requirements and appraisal ● All users must be up to date with MHRA and safety alerts with regards to medical products ● All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	HBVax Pro is indicated for the active immunisation of individuals at risk of exposure to hepatitis B virus, including healthcare workers, infants born to infected mothers, and individuals at occupational or lifestyle risk.
Inclusion criteria	- Individuals of any age who are at increased risk of hepatitis B infection. - Eligible under national immunisation or occupational health guidance. - Informed consent obtained. - No contraindications to the vaccine.
Exclusion criteria	- Known hypersensitivity to the active substance or any excipients. - Previous allergic reaction to any hepatitis B vaccine. - Acute severe febrile illness (postpone until recovered).
Cautions	- Use with caution in individuals with bleeding disorders or on anticoagulants. - Observe patient for 15 minutes post-vaccination. - Ensure adherence to the complete vaccination schedule. - Provide follow-up advice and written patient information leaflet (PIL).
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate

Details of the medicine

Name, form and strength of medicine	HBVax Pro
Legal category	POM
Storage	Store in a refrigerator (2°C–8°C). Do not freeze. Protect from light.
Route/method of administration	Intramuscular injection, usually in the deltoid muscle.
Dose and frequency	Adults and adolescents (16+): 10 or 20 micrograms per dose depending on age and risk group. Standard schedule: 0, 1, and 6 months. Accelerated schedule: 0, 1, 2, and 12 months.
Quantity to be administered and/or supplied	0.5 or 1 mL per dose depending on age and indication.
Maximum or minimum treatment period	Typically 6–12 months depending on schedule used.
Adverse effects	Injection site reactions (pain, redness, swelling), fever, fatigue, headache, nausea. Rare: hypersensitivity reactions. Refer to SmPC.

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: ● that valid informed consent was given ● name of individual, address, date of birth and GP ● name of healthcare practitioner ● name and brand of medication ● date of supply dose, form and route quantity supplied ● advice given, including advice given if excluded or declines treatment ● details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: ● Adults aged 18 years and over - keep records for 8 years ● Children aged under 18 years -keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

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